A PRIVATE BRIEFING, EL PASO, 2026-10-20

Six months of free decisions.

What to automate, what to buy, and what to leave alone before the new center opens.

Prepared for **Dr. Luis Ramos** and **Fernando Escarzaga**

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WHERE YOU ARE

Equipment is being signed. Real estate is being toured. No staff hired, no systems chosen.

Today

Every operational decision is cheap and reversible.

2027-01-01

Federal prior authorization APIs go live at the payers.

Target 2027-04

Your projected opening. Whatever is in the vendor contracts by then is what you live with.

Greenfield is the entire argument. Nothing on the slides that follow is possible to do this cheaply after opening.

CMS-0057-F, 89 FR 8758, compliance dates verified in the Federal Register rule text.

THE FREE MOVE

Put four things in every vendor contract before you sign. They cost nothing today.

- **API access** to your own data, named in the contract, not "available on request."
- **HL7 interface count and fee schedule** fixed at signature, not quoted per interface later.
- **Data export rights** in standard formats, on demand, at no charge.
- **DICOM query permissions** (worklist, query/retrieve, DICOMweb) for systems you authorize.

Before signature these are contract terms. After signature they are hostage terms.

THE HEADLINE NUMBER

11.5%

of denied prior authorizations were appealed

80.7%

of those appeals were partially or fully overturned

Four in five appeals win. Almost nobody files one.

Source: KFF, "Medicare Advantage Insurers Made Nearly 53 Million Prior Authorization Determinations in 2024," 2026-01-28, analyzing data insurers report to CMS. Medicare Advantage, all services; CMS does not report prior authorization data by service type.

WHY GREENFIELD MATTERS

40%

of physicians report having staff who work **exclusively** on prior authorization.

You have not hired that person yet.

Source: American Medical Association, 2025 AMA Prior Authorization Physician Survey, 1,000 practicing physicians (400 primary care, 600 specialists). Self-reported survey data.

PART ONE

The thing you did not ask about.

Prior authorization and eligibility verification. Scheduling, phones, and records are real costs. This one is larger, and it is the one with the best evidence.

WHAT IT COSTS IN TIME

13

hours per physician per week spent on prior authorization

40

prior authorization requests per physician per week

Of physicians who do not appeal denials, 59% say they do not expect to win and 52% cite insufficient staff time.

CMS used the same hours figure to price this burden at **220 million provider hours** over ten years, in the prior authorization rule itself.

Sources: American Medical Association, 2025 AMA Prior Authorization Physician Survey. Self-reported. CMS-0057-F, 89 FR 8758, 2024-02-08, Regulatory Impact Analysis; a regulatory projection, not measured savings.

THE DATES LAND ON YOUR OPENING

2026-01-01

Payers must decide prior authorizations in **7 calendar days** standard, **72 hours** expedited, and publish their metrics.

2027-01-01

Payers must operate a **FHIR Prior Authorization API**, plus Patient Access, Provider Access, and Payer-to-Payer APIs.

Target 2027-04

Your projected opening, into the new regime with no legacy process to unwind.

Who the rule reaches: Medicare Advantage, Medicaid, CHIP and exchange plans. Not employer-sponsored plans, TRICARE, or VA. The payers who move first set the standard the rest follow.

Almost 63% of medical plans report they are already building a FHIR prior authorization API ahead of the deadline.

Sources: CMS-0057-F, 89 FR 8758, compliance dates verified in rule text; covers Medicare Advantage, Medicaid and CHIP, and federally facilitated exchange plans. 63% figure: 2025 CAQH Index (DataSpring), 2026-02-19, payer self-report.

WHAT A PRIOR AUTH PROCESS ACTUALLY DOES ALL DAY

4.94%

of 17,913 outpatient MRI prior authorization requests were denied at one institution over four years. The other 95% were approved.

A process that approves 95% of what it touches is one you automate, not one you staff.

Source: Harrer S et al., J Am Acad Orthop Surg, 2025 Feb 15;33(4):e244-e252, PMID 39729599. Single institution, 2019 through 2022. The authors' own conclusion: the low denial rate "underscores the questionable utility of the prior authorization process."

PART TWO

The three things you did ask about, corrected.

Scheduling. Answering patients. Requesting records. All three are real. Two of them are not what they look like.

SCHEDULING: THE EMPTY SLOT IS NOT A NO-SHOW

22.3%

of outpatient radiology exams were cancelled

2.0%

were no-shows

70.2% of cancellations were patient-initiated, which means most empty slots are known in advance. That is a backfill problem, not a reminder problem.

Source: Aijaz A et al., "Sociodemographic Factors Associated with Outpatient Radiology No-shows Versus Cancellations," Acad Radiol, 2024 Aug;31(8):3406-3414, PMID 38705764. University of California, Irvine. 19,262 exams, 9,713 patients. Single center.

SCHEDULING: WHAT REMINDERS HONESTLY DO

5.1%

MRI no-show rate with phone reminders alone

3.8%

after adding a text message reminder

A real effect, 1.3 percentage points, in a randomized trial of 6,989 MRI patients. It did not improve arrival punctuality. **Your RIS will do this.**

Let it.

Source: Liu C et al., "Text Message Reminders Reduce Outpatient Radiology No-Shows But Do Not Improve Arrival Punctuality," J Am Coll Radiol, 2017 Aug;14(8):1049-1054, PMID 28583321. Massachusetts General Hospital. p = 0.02; only 67.5% of the texting arm could be reached by text.

ANSWERING PATIENTS: BUY IT, DO NOT BUILD IT

26.5%

of El Paso County residents aged 5 and over speak English less than "very well." 97.8% of them speak Spanish.

My recommendation: buy a healthcare voice vendor. I will write the specification, run the evaluation, and integrate and supervise it. Voice is the highest-blast-radius failure in this whole list, and it is a funded category I cannot beat on cost.

Source: American Community Survey 2024 5-year, Table B16004, El Paso County, population 5 and over of 813,194; 215,186 speak English less than "very well." Computed from raw ACS counts via Census Reporter.

The 30-day clock runs from receipt

Not from when the request reaches the right desk. One extension of up to 30 more days, with written notice inside the first 30. Outer limit 60 days.

Finding the study is unbillable

Fees may cover labor to copy, supplies, postage, and an agreed summary. Search and retrieval labor is explicitly not chargeable. The expensive half of the work cannot be charged for.

The harder problem is outbound. A new center opens with zero priors on file. Every comparison study comes from somewhere else, and RSNA is on record that DICOM exchange is not yet a TEFCA requirement.

Sources: 45 CFR 164.524 (right of access, deadlines, permitted fee components); Ciox Health v. Azar, 435 F. Supp. 3d 30 (D.D.C. 2020), which left the $6.50 flat-fee option in place; RSNA comment letter to ASTP/ONC, March 2026. OCR's Right of Access Initiative has produced more than 50 enforcement actions.

THE FIFTH CANDIDATE: THE NUMBERS THE OWNER ACTUALLY LOOKS AT

Studies by modality, by reader, by referrer. Turnaround. Scheduled versus completed. Revenue per slot.

All of it already lives in the RIS and the billing system. The problem is that they are two systems, and RIS reporting is built for radiology operations, not for an owner.

Smallest compliance envelope of the five. Cheapest to build. Luis's "am I being paid correctly for what I read" reconciliation is the same engine with a different view, and that one already has a customer.

THE SCORECARD

CANDIDATE	MY RECOMMENDATION	WHY
Prior authorization and eligibility	**Build**	Strongest evidence, vendors built for someone much larger, federal APIs arrive 2027-01-01.
Scheduling: reminders and self-scheduling	**Buy (the RIS)**	Commodity. Bundled in modern imaging RIS products.
Scheduling: cancellation backfill	**Build, small**	Ranked waitlist on top of the RIS. Addresses the 22.3%, not the 2.0%.
Answering patients (voice)	**Buy, I integrate**	Funded category, highest blast radius, Spanish bar is unforgiving.
Records: inbound requests, outbound priors	**Build, selectively**	Hard deadlines, unbillable retrieval labor, unsolved outbound gap.
Owner reporting	**Build**	Custom by nature. Smallest compliance footprint.
Billing, claims, and the eligibility feed	**Buy**	Commodity clearinghouse function. I evaluate reachability and contract terms, not the claims work. Row 1 reads the feed; it does not replace the vendor.

PART THREE

Proof: RadAssist.

Browser-based structured radiology reporting, with calcium score and thyroid modules. Built by me. Used daily on real patients by the radiologist sitting at this table.

radassist.app

RADIOLOGY REPORT ASSISTANCE

The agent drafts. The radiologist signs. Nothing clinical leaves without a human signature.

The report: RadAssist, today

Luis uses it daily. It reads the scanner's numbers from a screen capture, drafts the report from his template, and he confirms every value before it applies. At your center the path has a name: DICOM worklist and query for the study, HL7 v2 ORU to carry the signed report into the RIS.

The records: a different build

Requesting records runs the other way. The paths: DICOM query and retrieve where a facility grants it, a request out and DICOM import where it does not, and the payer Provider Access API in 2027 for the claims record, in-network only, no images.

One instance per center

Not one shared stack. My recommendation, and it costs more, not less. Each center runs its own instance: its own database, credentials, and audit log. No patient record from one center is readable from another. The reason is governance: separate principals, separate books, a clean line any auditor can see. Share a back office if you choose; the specification is written once either way. The data stays apart.

Which did you mean?

Two different builds, specified separately. I would rather ask than guess.

BOUNDARIES: THE FDA QUESTION, ANSWERED BY THE STATUTE

> Software intended "for administrative support of a health care facility, including the processing and maintenance of financial records, claims or billing information, appointment schedules, business analytics, information about patient populations, admissions, practice and inventory management, ... determination of health benefit eligibility, population health management, and laboratory workflow" is not a medical device.

Every candidate on the scorecard is on that list by name. The agents stop at the clinical boundary: nothing interprets an image, selects a protocol, or recommends a study.

Source: 21 U.S.C. 360j(o)(1)(A), read verbatim at Cornell Legal Information Institute. 360j(o)(2) further provides that FDA "shall not regulate" excluded administrative functions even when they coexist with device functions.

Texas SB 1188

Electronic health records with Texas patient data must be physically stored in the United States as of 2026-01-01. Every component, including the model provider, gets verified for US residency before anything is promised.

TCPA, wireless healthcare exemption

One message per day, three per week, per patient. No billing or financial content. Provider name and opt-out in every message. $500 per violation, up to $1,500 if willful.

The AI vendor's BAA

Anthropic's agreement now covers Skills, Web Search, Memory, and the Bash and Text Editor tools. It still excludes Computer Use and Web Fetch, which are exactly what you would reach for to drive a payer portal with no API. So the prior auth design routes through APIs, EDI, or a human step. Not screen-driving.

Texas HB 300

Its definition of covered entity is broad enough to reach me directly, not only through your BAA. I carry that obligation in my own right.

Sources: Texas SB 1188 (89R), signed 2025-06-20, effective 2025-09-01, bill history at capitol.texas.gov; 47 CFR 64.1200(a)(9)(iv) and 47 U.S.C. 227; Anthropic BAA documentation at privacy.claude.com, accessed 2026-10-01; Texas Health and Safety Code Chapter 181.

THE QUESTION YOU ARE GOING TO ASK

What happens when I get hit by a bus?

- **You own the specification.** It is written for a competent engineer who has never met me, and it is the deliverable of the design engagement whether or not I build anything.
- **You own the code.** Everything specific to this center lives in a repository under your organization's account, with my access revocable by you. Existing product of mine that I extend for you, you hold under a perpetual license with the source in your hands.
- **Runbooks, not memory.** Every operational procedure is written down: how it runs, how it fails, who to call at each vendor.
- **No proprietary runtime.** Standard interfaces (HL7 v2, DICOM, payer APIs) and a named model vendor under its own BAA. Any integrator can take it over.
- **Buy where buying is right.** The voice vendor and the RIS are yours directly. Nothing routes through me that does not have to.

THE MOST IMPORTANT SLIDE

What I recommend you do not buy from me.

- **A custom voice agent.** Buy a vendor. I will specify, evaluate, and integrate it, and I will tell you when it is not good enough in Spanish.
- **Reminders, self-scheduling, and intake forms.** Your RIS does this. Make sure the contract says so, then use it.
- **Anything built to resell to other centers.** This center is the reference implementation and the priority. If I ever take on a second site, your service terms do not change, in writing.
- **Anything that touches the image clinically.** Not from me, not from anyone, without an FDA pathway.

IF IT WERE MY MONEY

I would do three things before signing another vendor contract.

- Put the four access terms into every contract this month. It is free.
- Decide now whether the prior authorization desk is a hire or a system. That decision expires the day you hire for it.
- Tell me what front desk and prior authorization actually cost you last time. Your number beats any benchmark I could bring.

And one question for you: what would you do differently this time, knowing how the last one ended up?

THANK YOU

Every one of these decisions is reversible today. None of them will be in six months.

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APPENDIX

Scope and investment.

Reference, after the decisions. The case above stands whether or not you turn this page.

WHAT I AM PROPOSING

A design engagement over your six-month build. Thirty thousand, in two phases, with a gate you control.

Phase 1 · $12,000 · first 90 days

- Vendor contract language for the four access terms.
- RIS, PACS, and billing vendor evaluation against the automation plan.
- Voice vendor specification and shortlist.
- Everything that has to happen *before* you sign a vendor.

Phase 2 · $18,000 · back 90 days

- Written operational specifications: prior auth, records, cancellation backfill, owner reporting.
- Payer-specific prior authorization design once your payer mix is known.
- Build-ready. You own it.

At ninety days you decide whether Phase 2 goes. The build is priced from the specification after you open, with real volumes in hand.